

ENGLISH FOR WORK / SEPTEMBER 2026

Pharmaceutical English

Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For clinical development, regulatory affairs, pharmacovigilance, quality, CMC, manufacturing, medical affairs, market access, and compliance teams.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

Accelerated approval

Approval pathway using a surrogate or intermediate clinical endpoint reasonably likely to predict clinical benefit.

AE

Adverse event; unfavorable medical occurrence after product use, regardless of causality.

API

Active pharmaceutical ingredient: the component intended to provide a medicine's pharmacological activity.

Batch record

Documented record of manufacturing steps, controls, and results for a batch.

Biosimilar

Biologic highly similar to a reference product with no clinically meaningful differences.

BLA

Biologics license application requesting approval to market a biologic.

Blinding

Keeping treatment assignment unknown to reduce bias.

CAPA

Corrective and preventive action addressing root cause and recurrence prevention.

Case narrative

Written summary of an individual safety case.

CMC

Chemistry, manufacturing, and controls information supporting product quality.

Complete response letter

FDA communication that an application is not ready for approval in its current form.

Confidence interval

An interval produced by a method with a stated long-run coverage rate under its assumptions.

Control arm

Comparator group used to interpret the effect of the investigational treatment.

Data integrity

Completeness, consistency, accuracy, and reliability of data throughout the lifecycle.

Deviation

Departure from approved procedure, process, specification, or expected result.

Estimand

Precise description of the treatment effect being estimated.

Exploratory endpoint

Outcome assessed to generate hypotheses or additional insight, usually not definitive.

Fair balance

Balanced communication of benefits and risks so the message is not misleading.

Formulary

List of medicines covered or preferred by a payer or health system.

HEOR

Health economics and outcomes research.

IND

Investigational new drug application allowing clinical investigation in humans in the United States.

Indication

Disease, condition, or patient population for which a product is intended or approved.

Indication statement

Approved language describing what the drug is approved to treat and in whom.

Informed consent

Process and documentation showing that participants understand key trial information before participation.

Intercurrent event

Event after treatment initiation that affects interpretation or existence of measurements.

Lifecycle management

Post-approval strategy for evidence, indications, formulations, access, safety, and competition.

MedDRA

Medical Dictionary for Regulatory Activities used to code medical terms.

Medical information

Function that responds to medical inquiries with approved, balanced, evidence-based information.

MLR

Medical, legal, and regulatory review of materials.

MOA

Mechanism of action; how the product is understood to produce a biological effect.

NDA

New drug application requesting approval to market a drug.

Off-label

Use outside the approved product labeling; the applicable legal and professional context must be distinguished from promotional activity.

OOS

Out of specification result requiring investigation.

P-value

Measure of compatibility between observed data and a null hypothesis under model assumptions.

Prescribing information

Approved product information describing indication, dosing, warnings, adverse reactions, and other use information.

Primary endpoint

Main outcome measure used to evaluate the primary objective.

Prior authorization

Payer requirement for approval before coverage.

Process validation

Evidence that a manufacturing process consistently produces product meeting quality requirements.

Promotional claim

Product-related statement intended to promote use and requiring support and compliance review.

Protocol

Document describing study objectives, design, population, assessments, endpoints, safety, and analysis.

Protocol deviation

Departure from the approved protocol or study procedures.

Randomization

Assignment to treatment groups by chance to reduce bias.

REMS

Risk Evaluation and Mitigation Strategy used to manage serious known or potential risks when required.

Risk-based monitoring

Monitoring approach focused on critical risks to participant safety and data reliability.

Risk-benefit

Integrated assessment of benefits, risks, uncertainty, disease severity, and alternatives.

RWD

Real-world data routinely collected from health care or patient sources.

RWE

Clinical evidence about usage, benefits, or risks derived from RWD analysis.

SAE

Serious adverse event meeting criteria such as death, life-threatening event, hospitalization, disability, or birth defect.

Scientific exchange

Non-promotional scientific communication, usually handled within medical affairs boundaries.

Secondary endpoint

Additional outcome measure supporting other objectives.

Sensitivity analysis

Analysis testing how robust results are to assumptions or data handling choices.

Signal detection

Process for identifying information that may suggest a new or changed risk.

Specification

Quality standard a material or product must meet.

Step therapy

Payer rule requiring use of one therapy before another.

SUSAR

Suspected unexpected serious adverse reaction.

TPP

Target product profile describing desired product attributes and evidence needs.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Match certainty to evidence

The available evidence shows

This may indicate ..., but

We have not yet established whether

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Transfer information and responsibility

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Give a useful status update

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

Disagree with a useful reason

I understand the goal. My concern is

That statement goes beyond

Could we ... before confirming ...?

Acknowledge the goal without pretending agreement. Name the specific problem and its implication, then ask a focused question or propose a next step. Be direct when a safety or ethical boundary requires it.

Make a complex point clear

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Drug Development Strategy, Unmet Need, TPP, and Evidence Logic

A development slide describes an early laboratory result as evidence that a product will benefit patients. Clinical benefit has not been established.

The result supports further investigation in this laboratory setting. It does not establish patient benefit. I'll revise the wording to distinguish the early finding from the clinical questions still to be tested.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

02 | Regulatory Pathways, IND Readiness, NDA/BLA Strategy, and Agency Interaction

A team is preparing a question for a regulatory meeting. The draft asks broadly whether the whole development plan is acceptable.

Could we identify the specific issue, proposed approach, and evidence supporting it? A focused question will make the requested feedback clearer than asking for general approval of the plan.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

03 | Clinical Trial Design, Protocols, GCP, and Operations

Two trial documents give different visit-window descriptions. A coordinator is asked which one to use but has not confirmed the controlling version.

The documents appear inconsistent. Which approved version governs this activity? I'll raise the discrepancy through the study process and request clarification before communicating a definitive instruction.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

04 | Endpoints, Estimands, Statistics, Data Readouts, and Clinical Meaning

A trial slide highlights a secondary endpoint while the primary endpoint result needs explanation. The audience may mistake the highlighted result for the main study conclusion.

Let's state the primary endpoint result clearly and identify the secondary analysis as such. The presentation should explain what each result supports and any limits on interpretation.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

05 | Pharmacovigilance, Safety Signals, Risk-Benefit, and Label Updates

A team member receives a report of a possible adverse event. Key details are incomplete, and the team has a defined safety-reporting process.

I've received a report that needs review through the safety process. Here are the details available and the information still missing. Please confirm receipt; I have not determined causality or classification.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

06 | CMC, CGMP, Quality Events, Manufacturing, and Supply

A quality review identifies missing documentation in a batch record. A manager asks whether the batch can be described as fully reviewed.

The documentation review is still open. I'll report the missing items and their review status. We should not describe the batch review as complete until the responsible quality team confirms it.

Try it again: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

07 | Labeling, Medical Affairs, Promotion Review, and Compliance Boundaries

A draft promotional slide uses stronger benefit language than the approved material supplied for review.

The proposed wording goes beyond the material we have been asked to use. Could we revise it to match the approved claim and send the change through the appropriate review process?

Try it again: The other person repeats the request more firmly. Restate the specific concern calmly and explain the appropriate next route.

08 | Market Access, RWE, Launch Readiness, Biosimilars, and Lifecycle Management

A market-access presentation combines trial findings and observational data without identifying the different study designs.

Let's label the evidence sources separately. The trial and observational study answer different questions and have different limitations. That distinction will help the audience interpret the findings accurately.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.